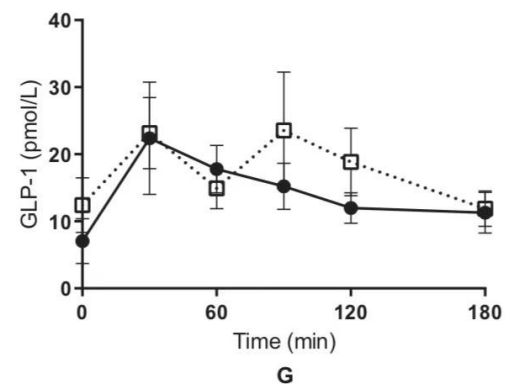
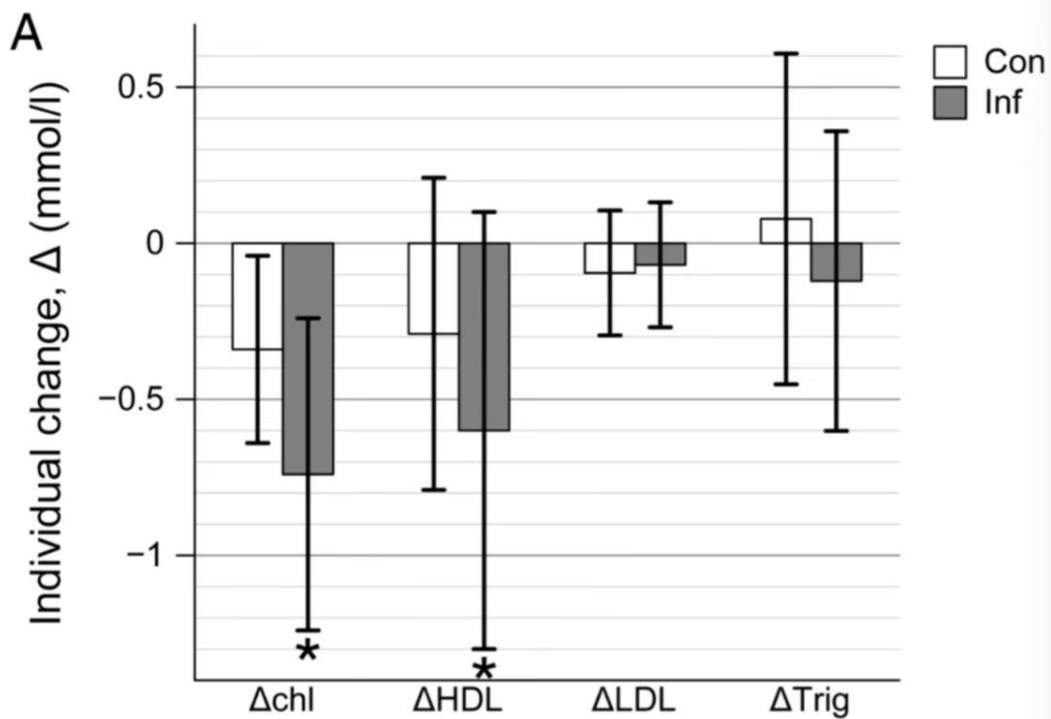
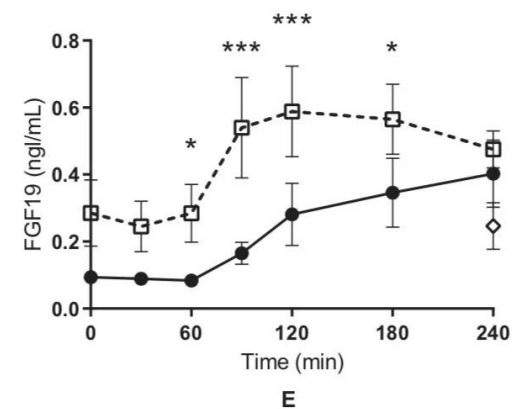
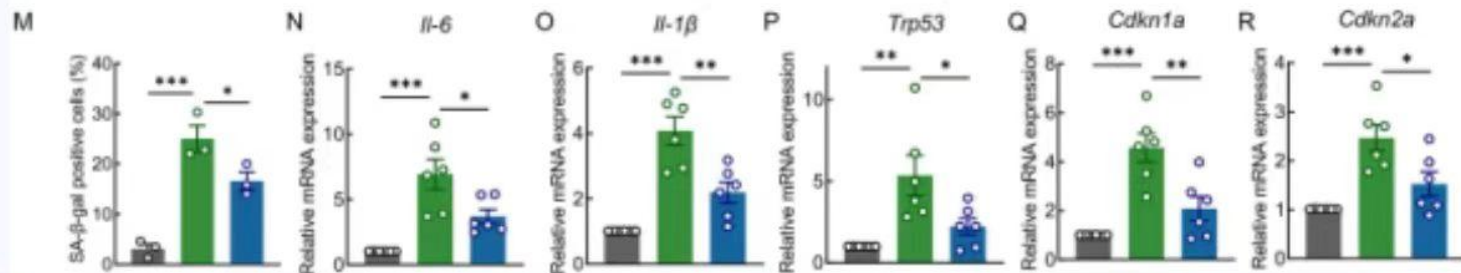
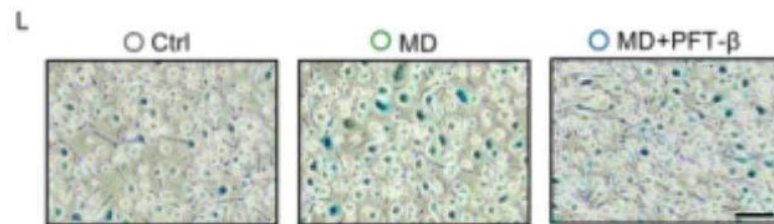
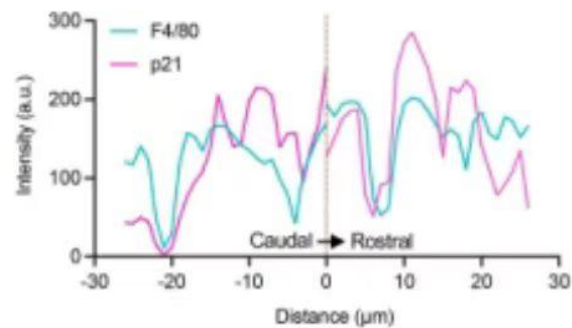




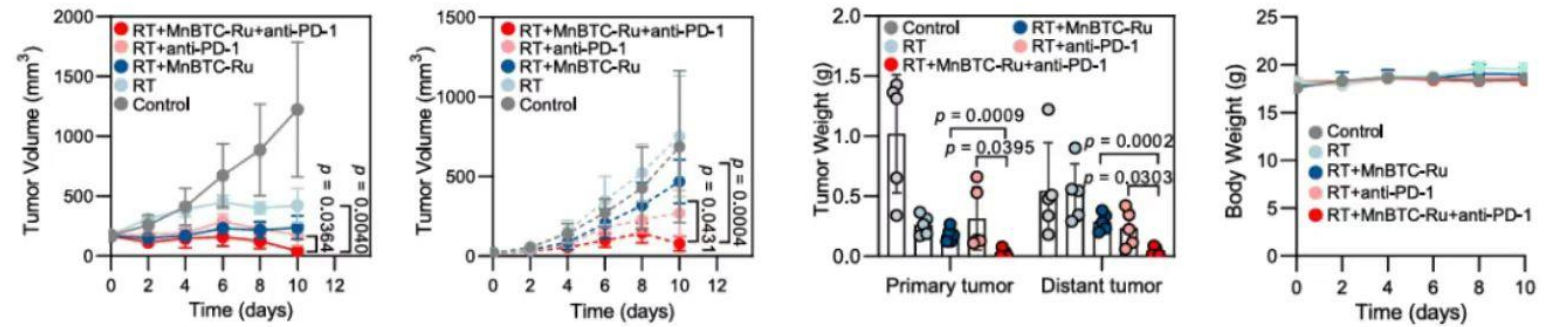
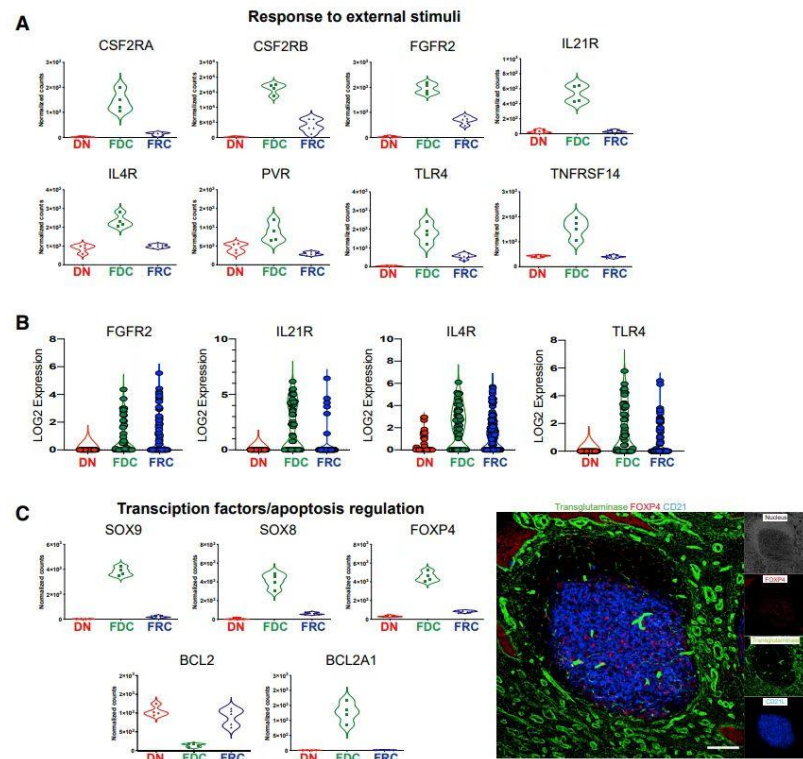
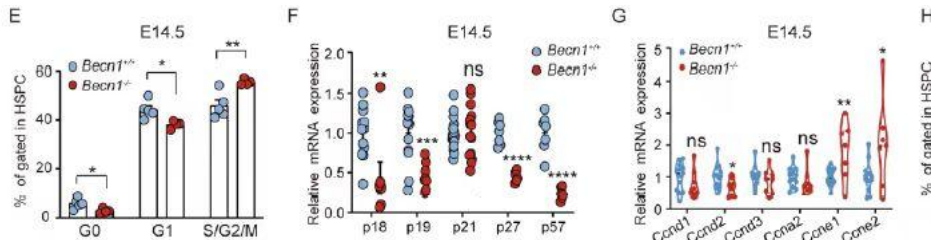
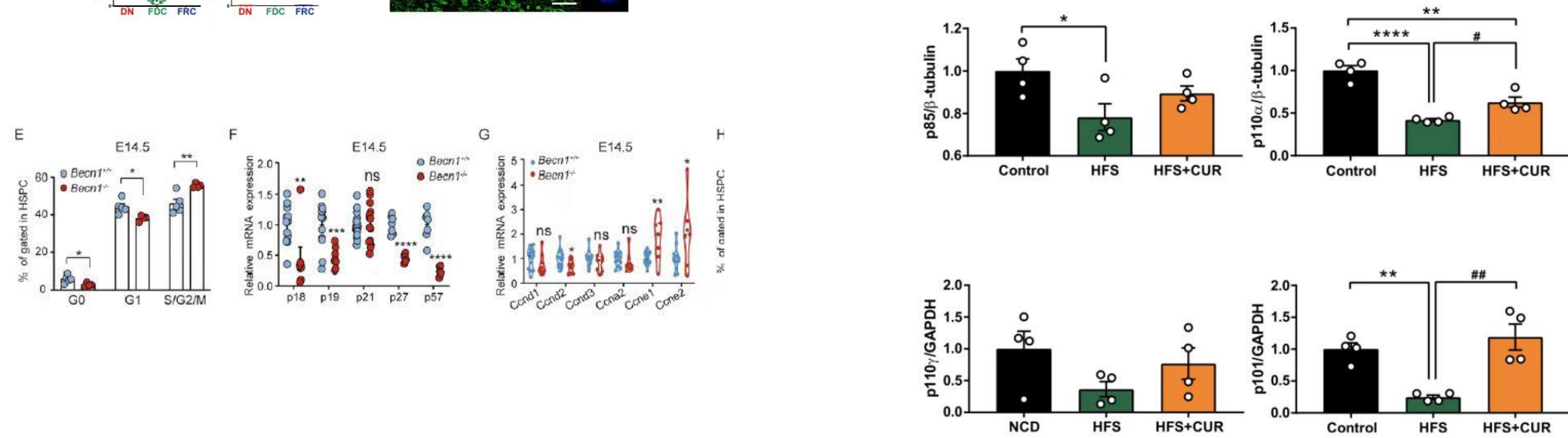
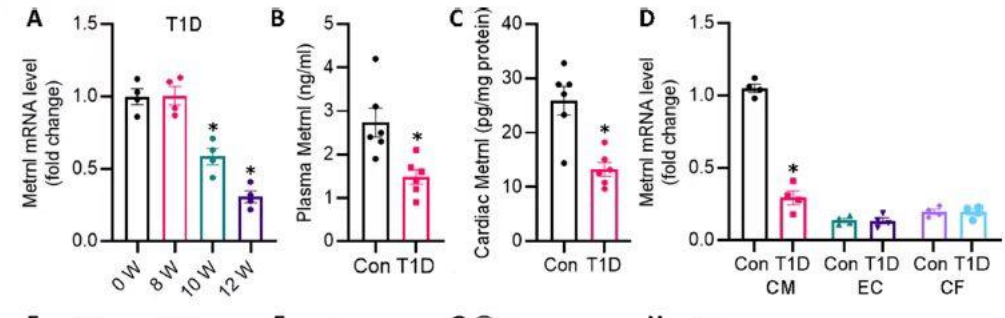
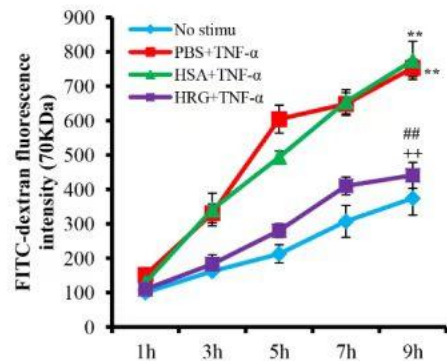
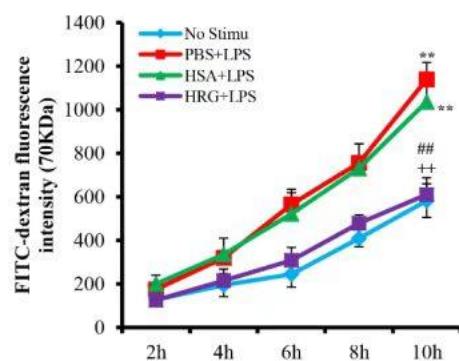


Figure 1. RT+MnBTC-Ru+anti-PD-1

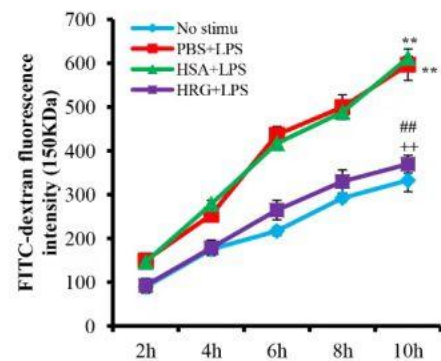
Figure 2. RT+MnBTC-Ru+anti-PD-1



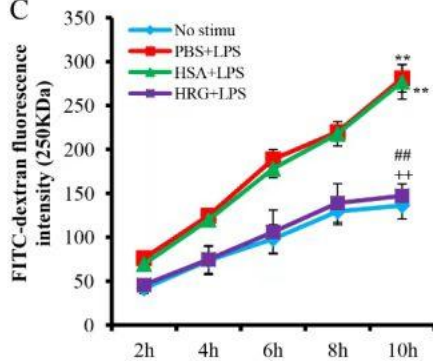
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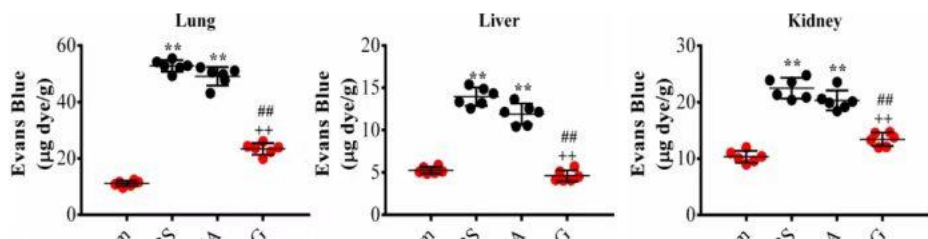
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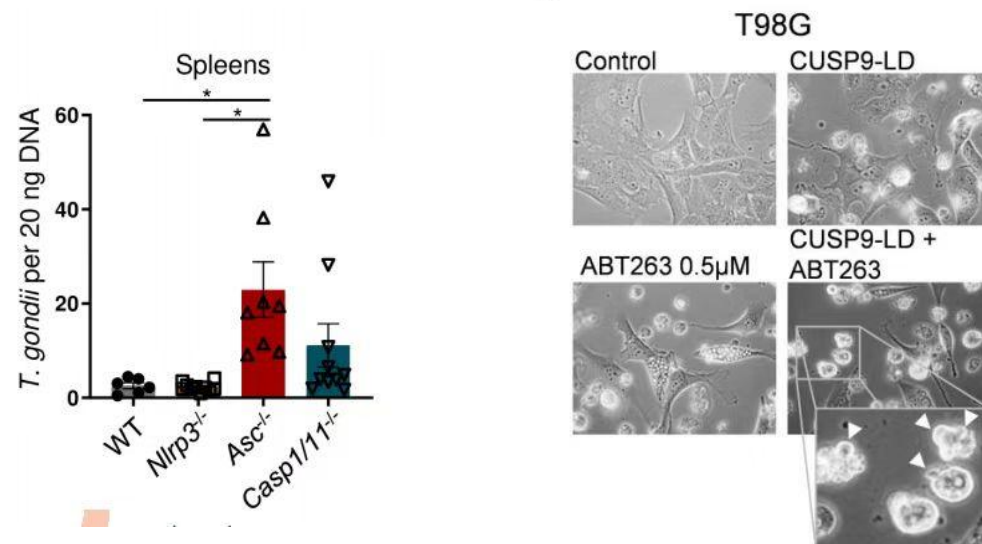
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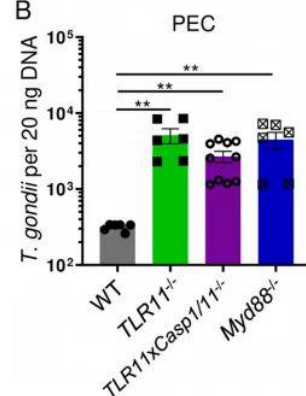
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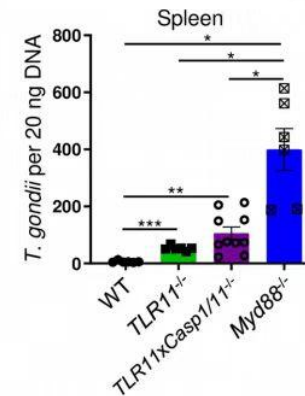
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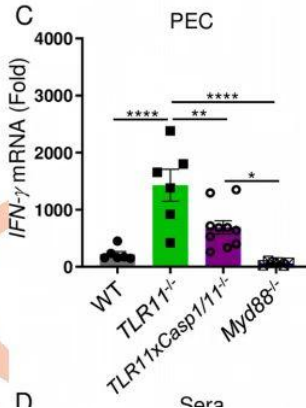
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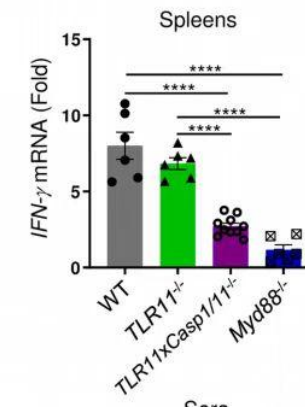
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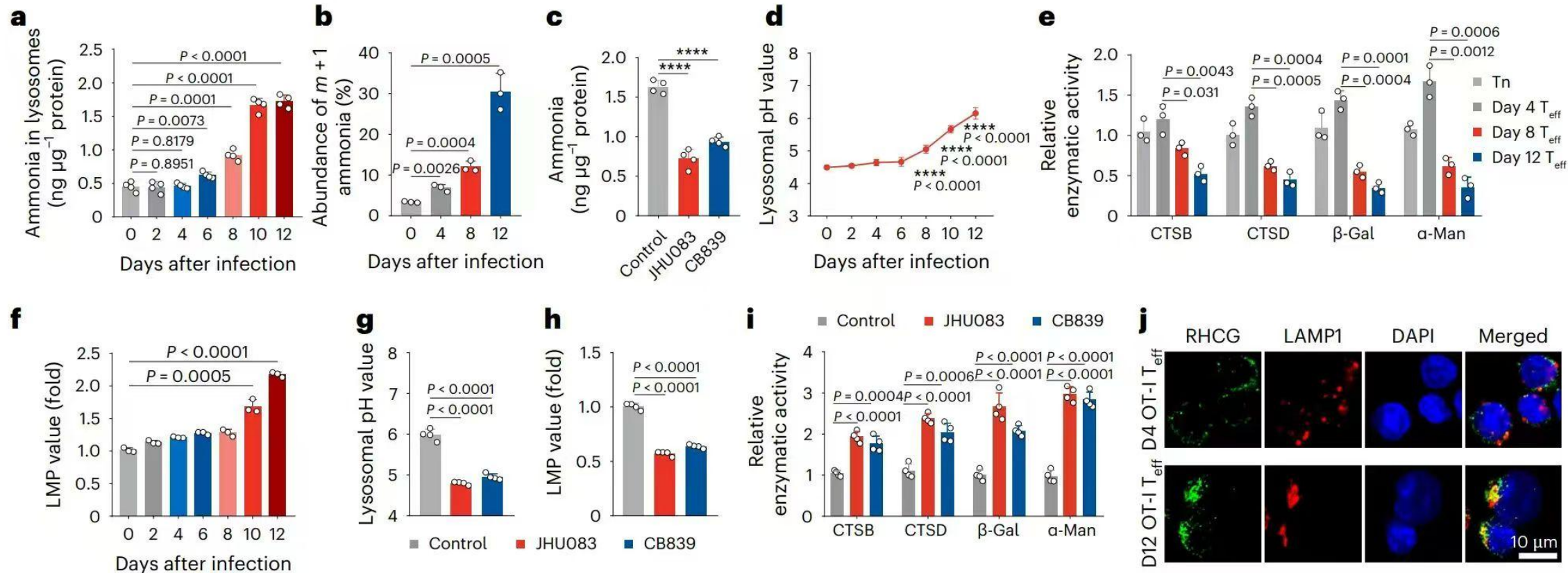


C



C





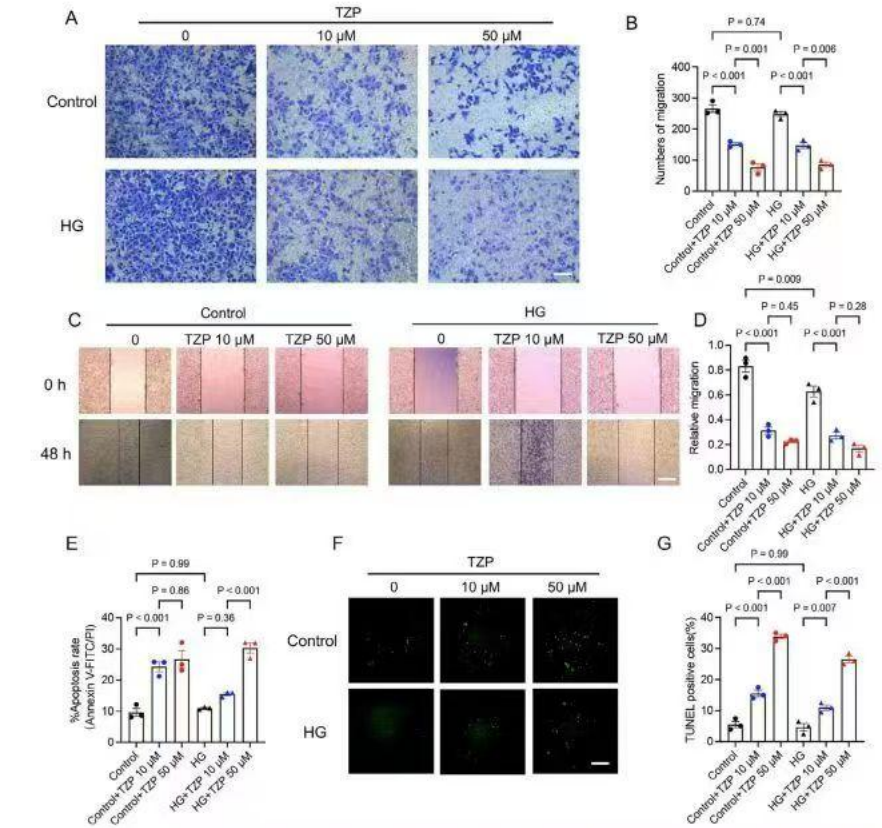
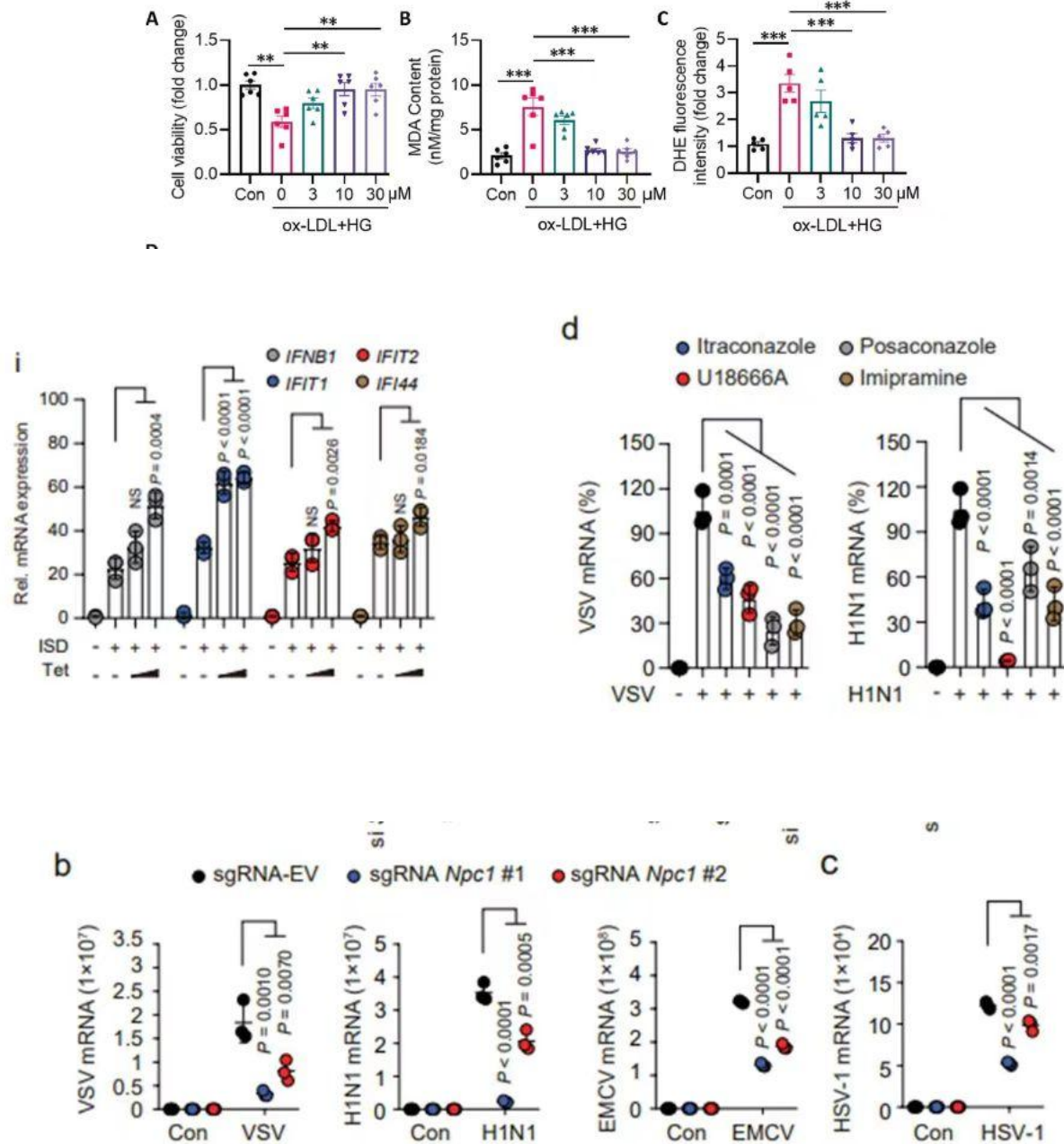
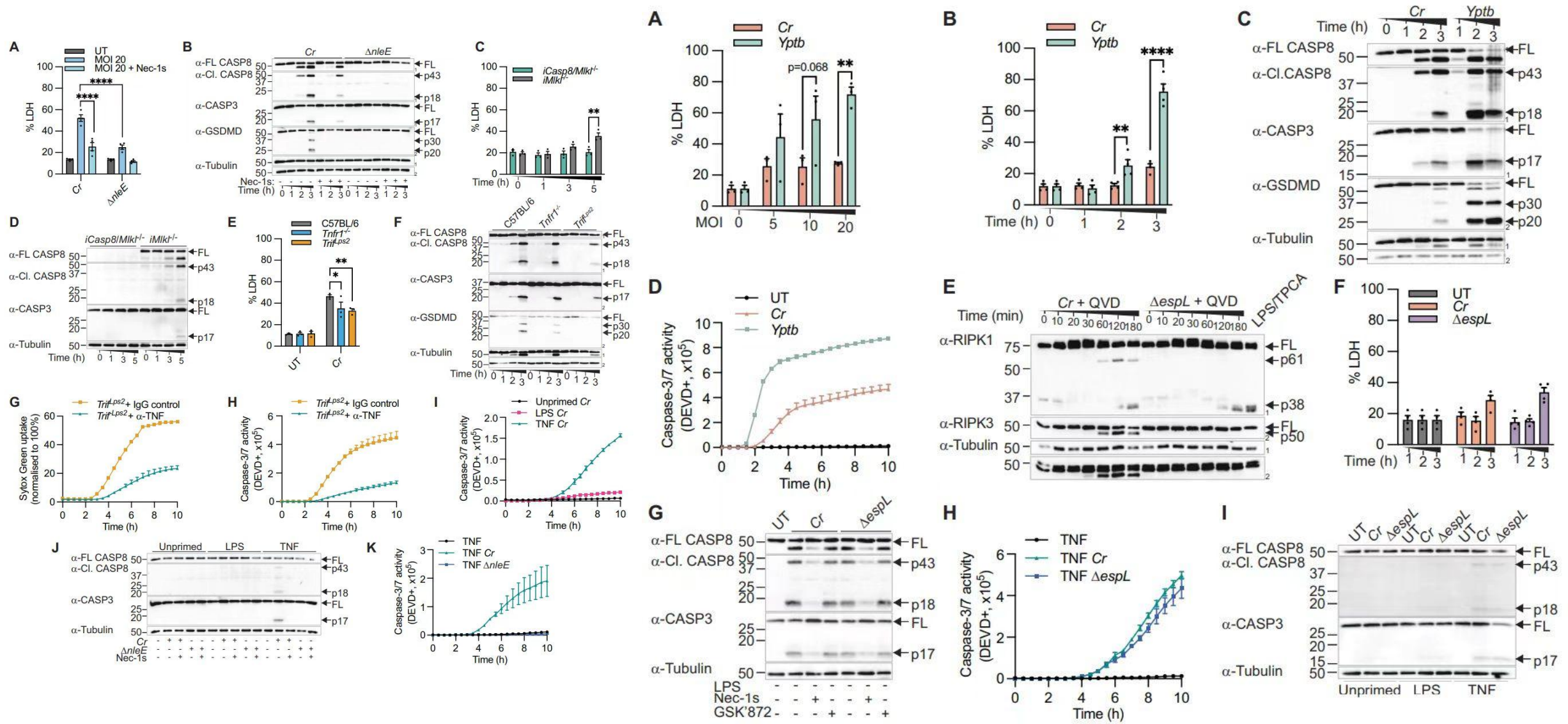
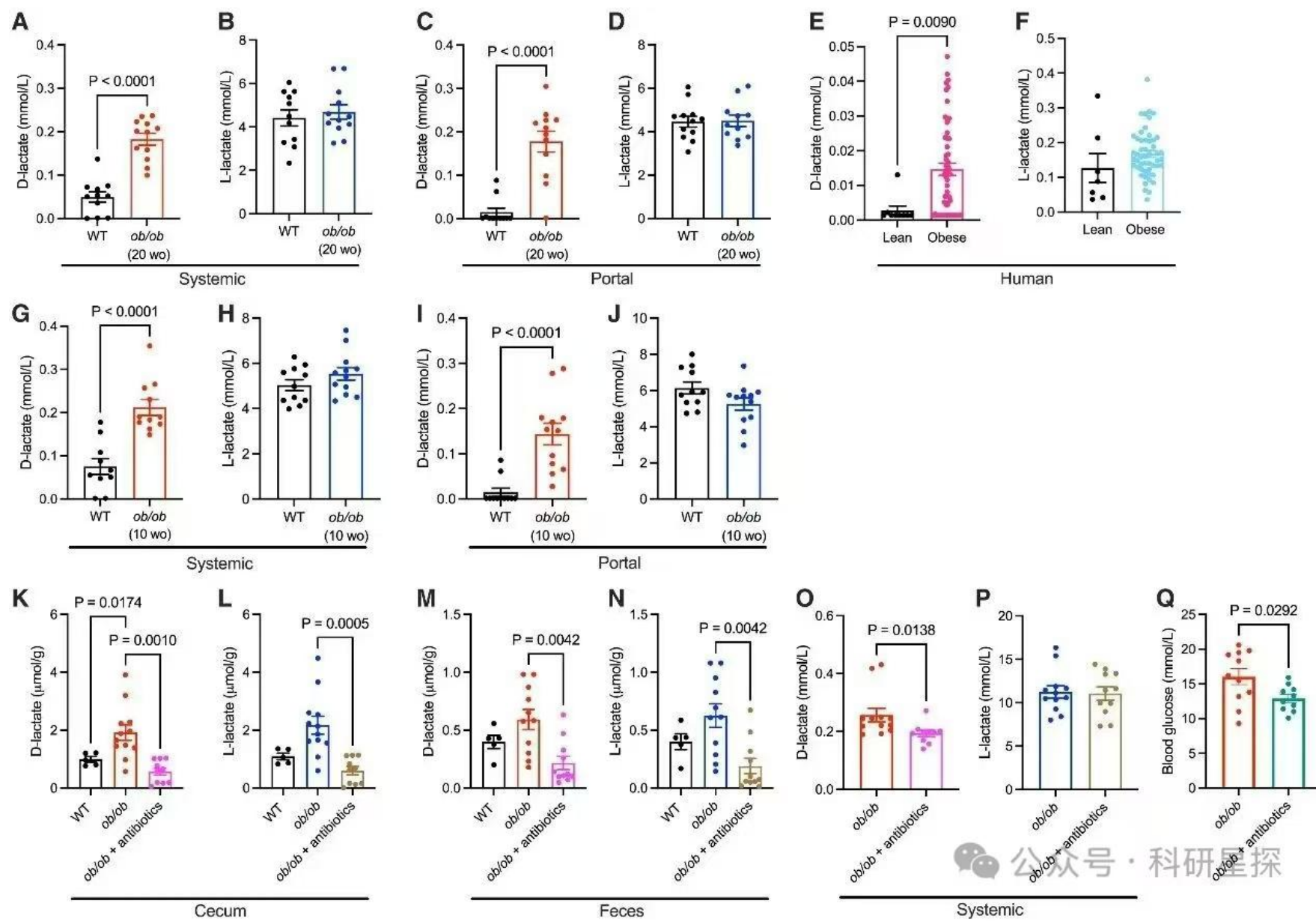
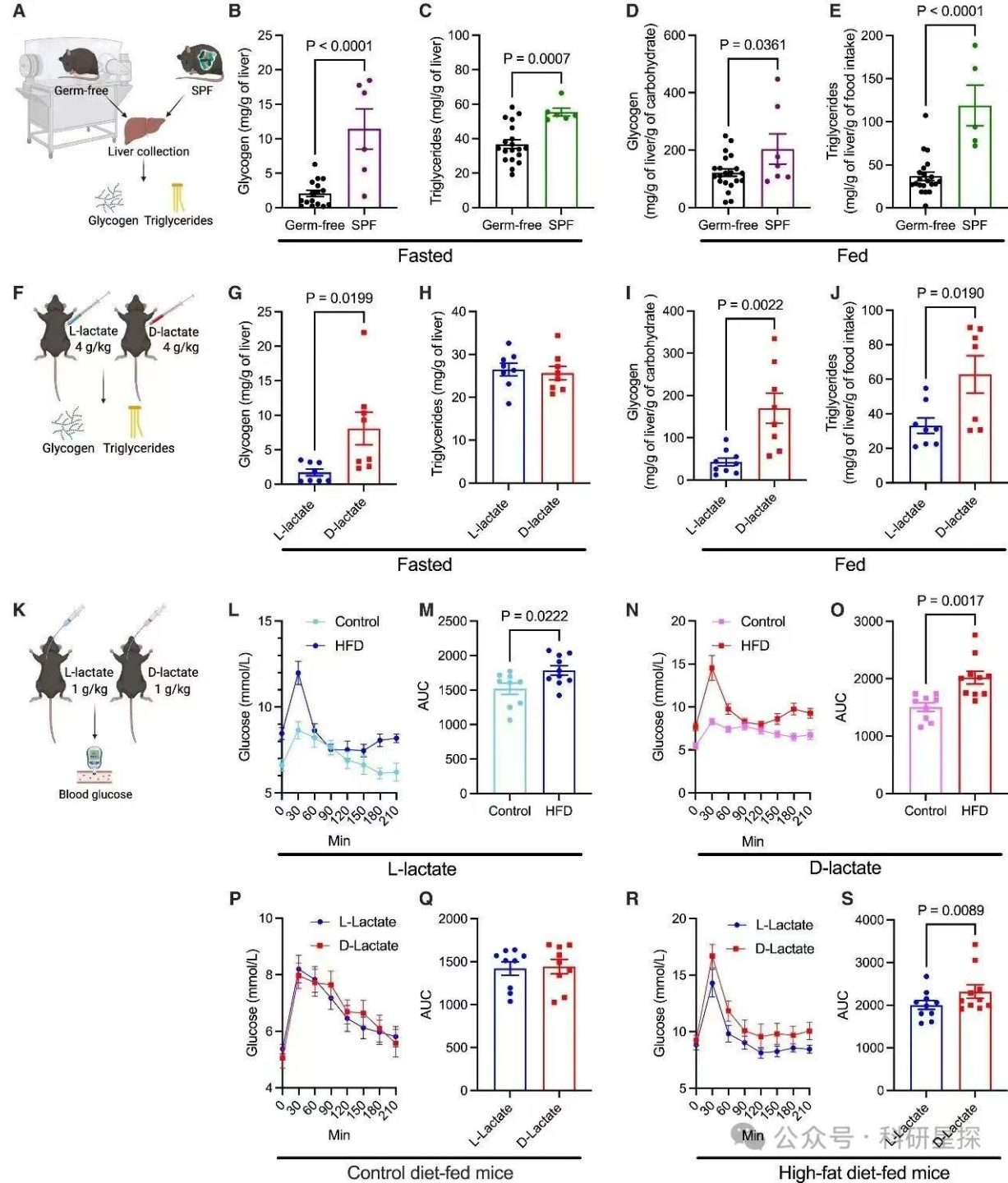
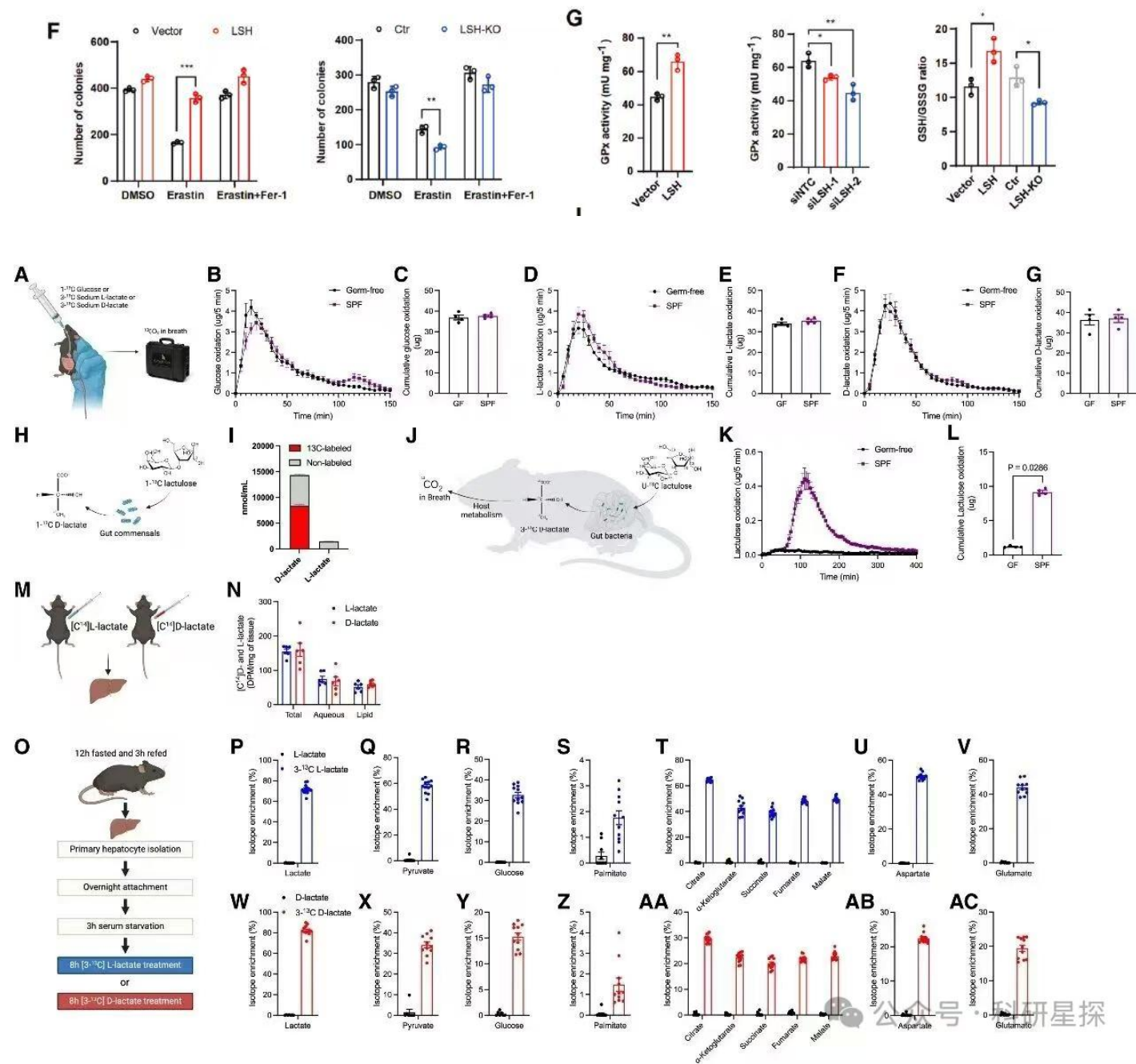


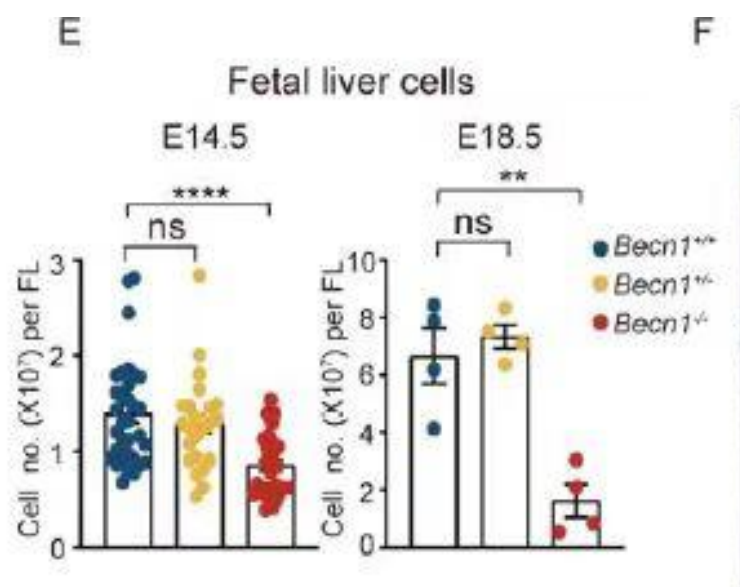
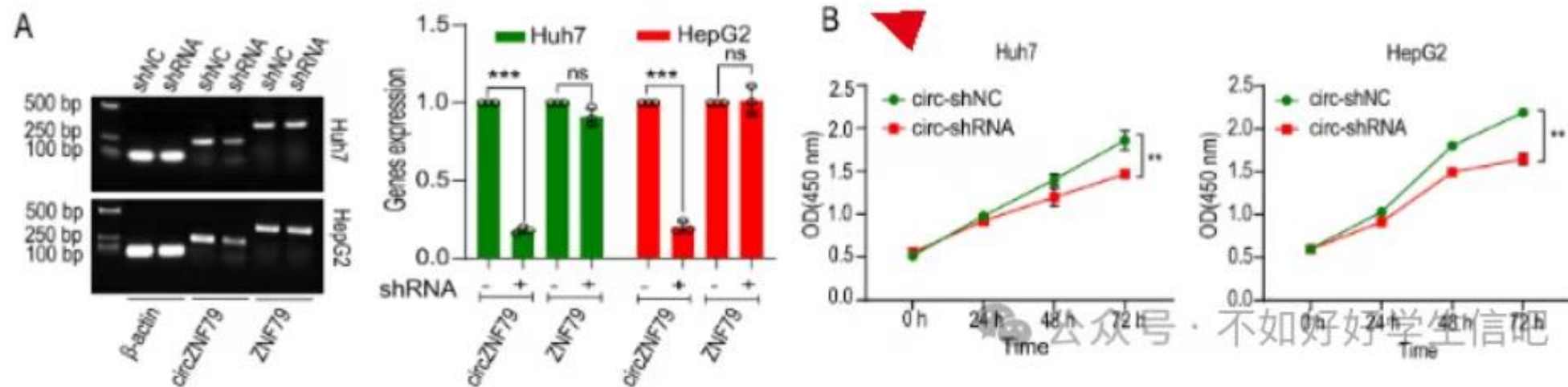
Figure 1. TZP inhibited CRC cell growth and migration under hyperglycemic and non-hyperglycemic conditions in vitro. MC38 cells with normal glucose (2 mM) (Control group) or high glucose (33 mM) (HG group) were treated with TZP in 10 or 50 μ M for 48 h. Cancer cell migration (invasiveness) and apoptosis were then measured. (A-B) Migration/invasion was evaluated by the transwell assay ($n = 3$) (Scale bar: 200 μ m). (C-D) Migration was assessed by the wound healing assay ($n = 3$) (Scale bar: 200 μ m). (E) Cells were stained with specific V-EIT and PI for flow automatic analysis ($n = 3$).

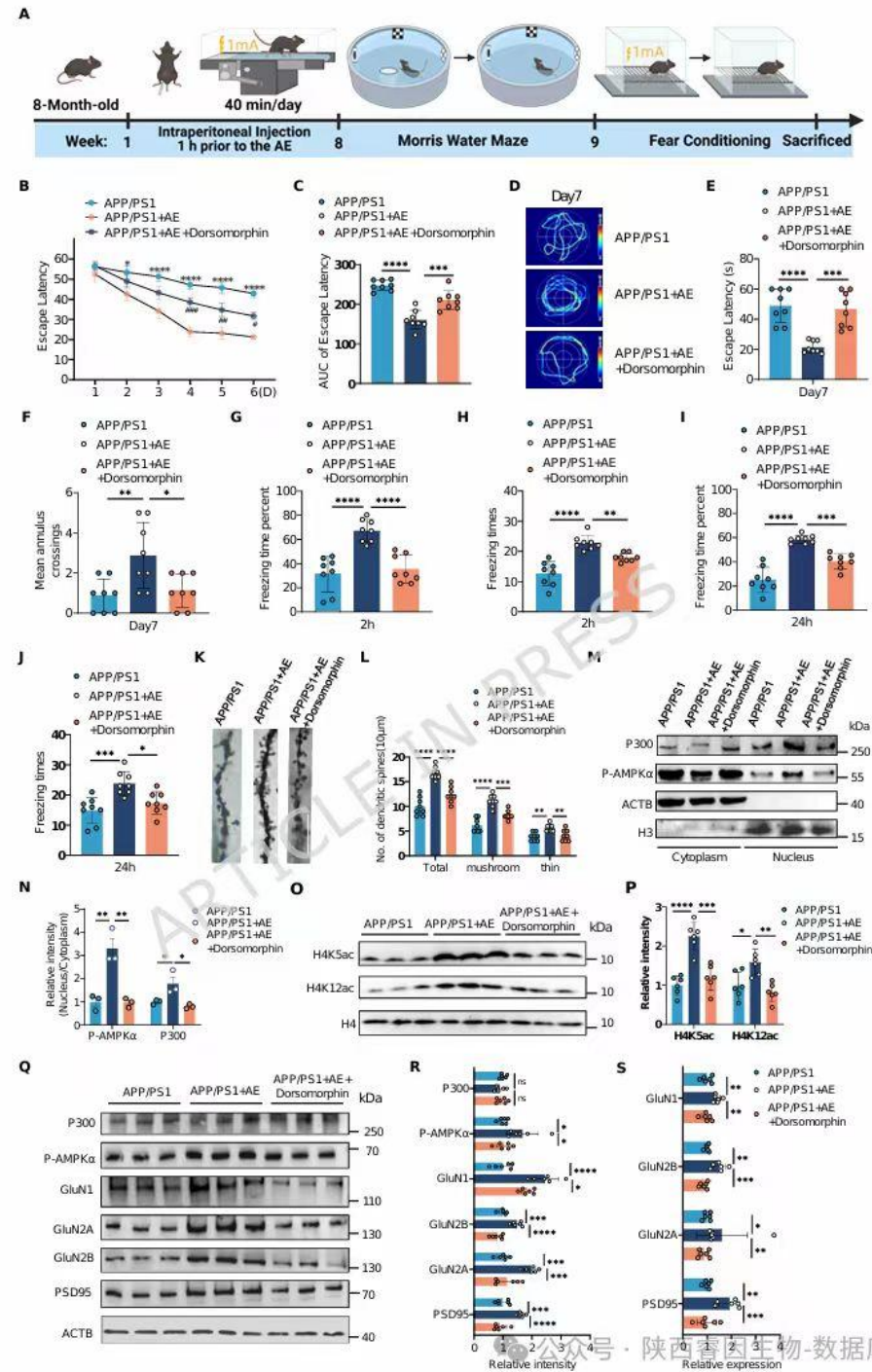
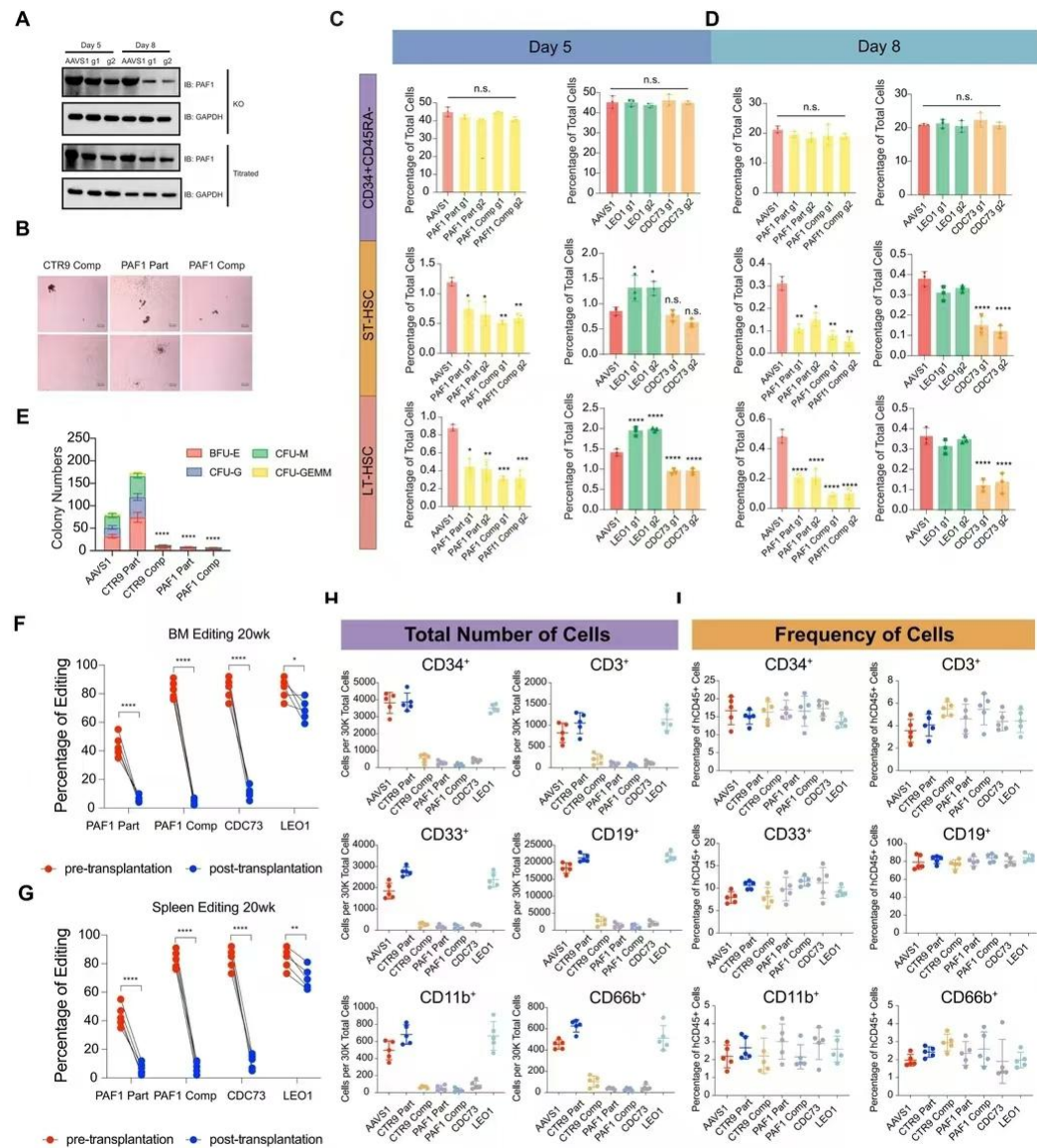


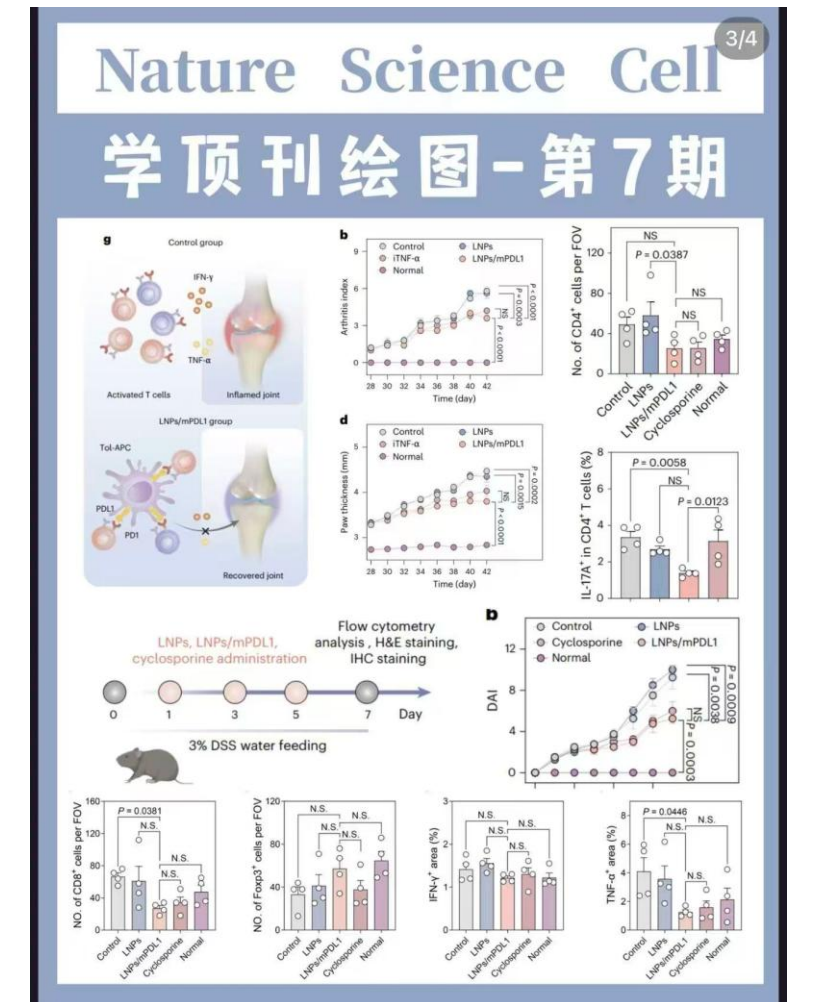
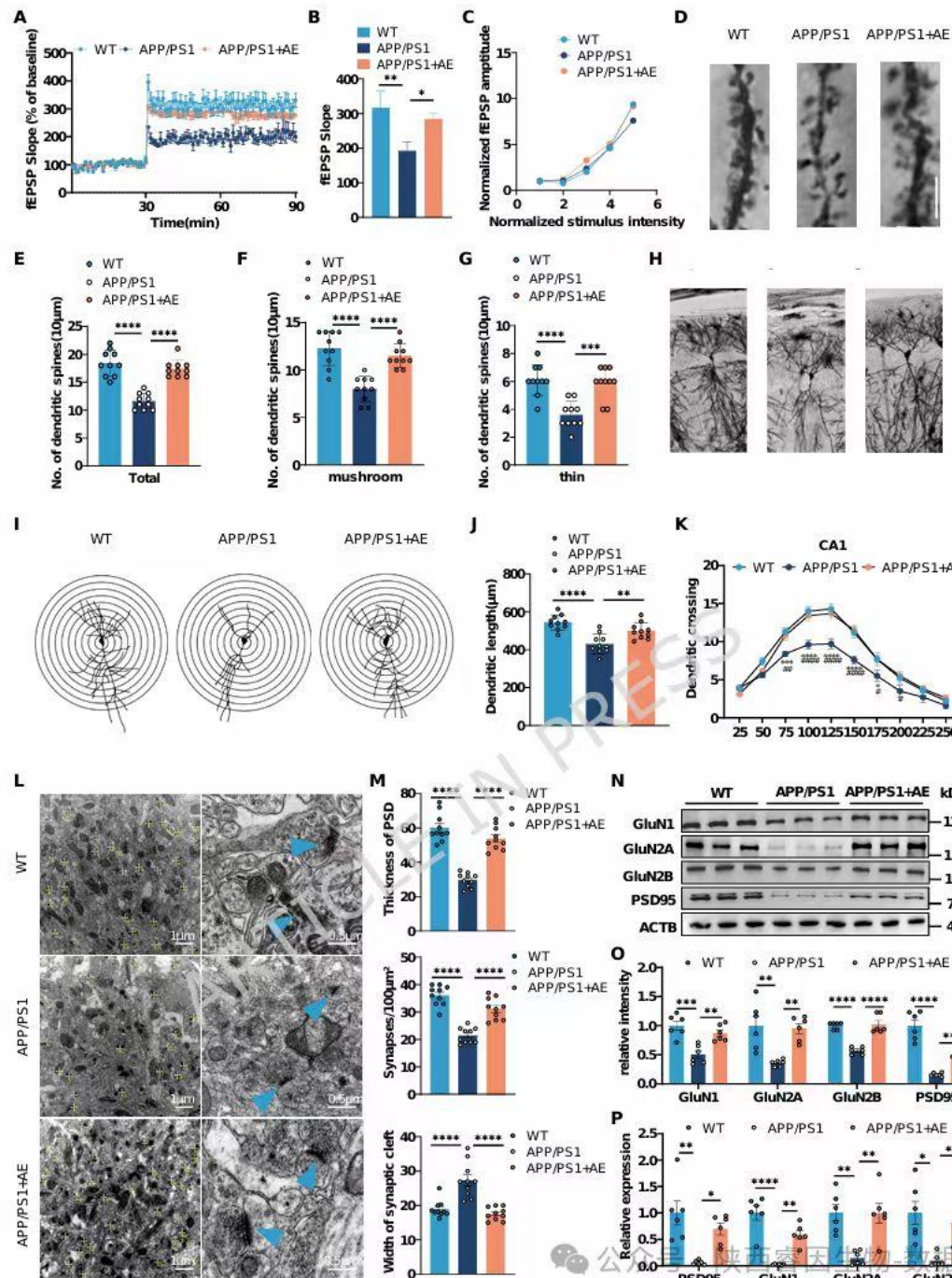


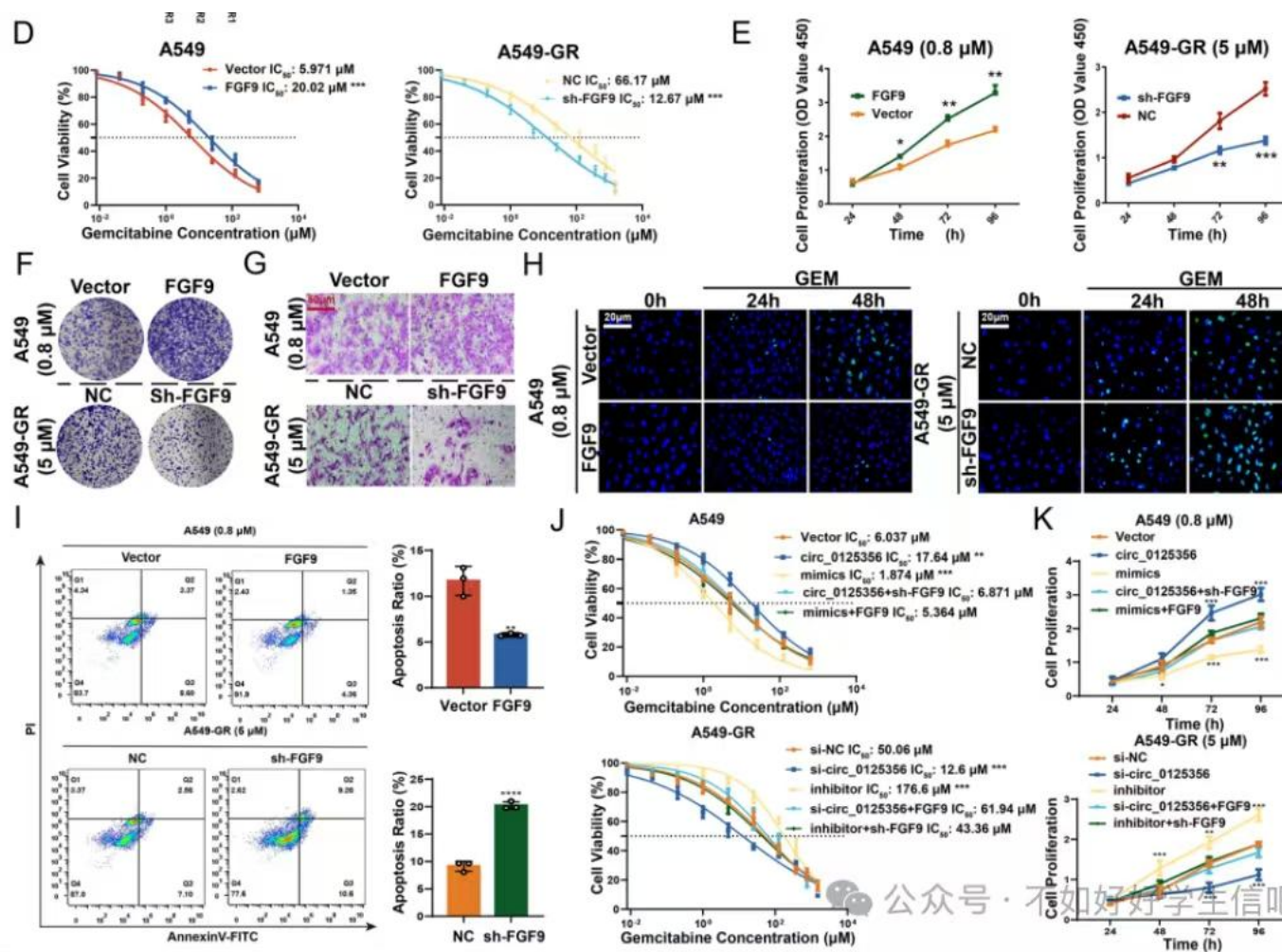
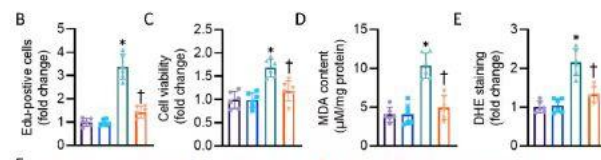


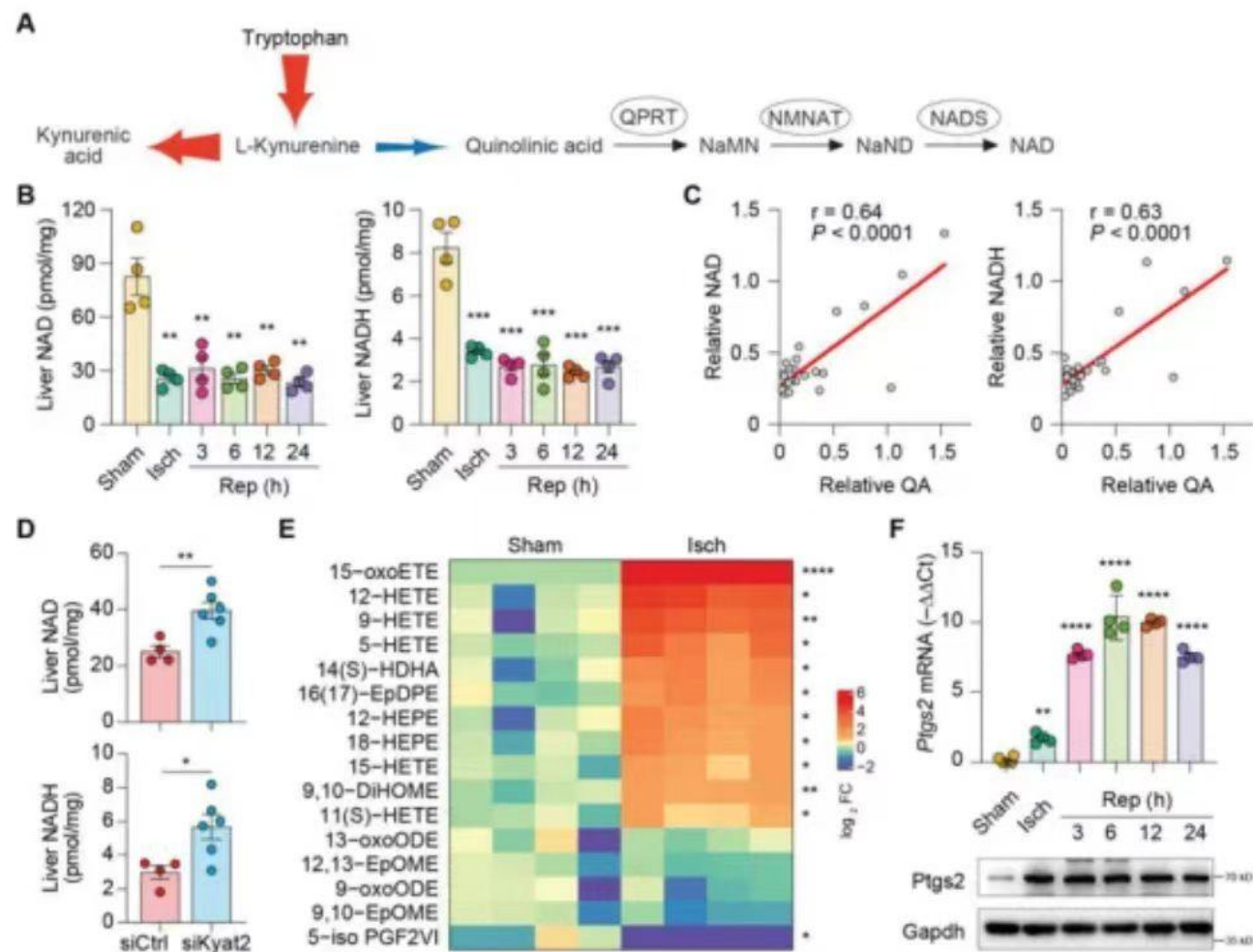


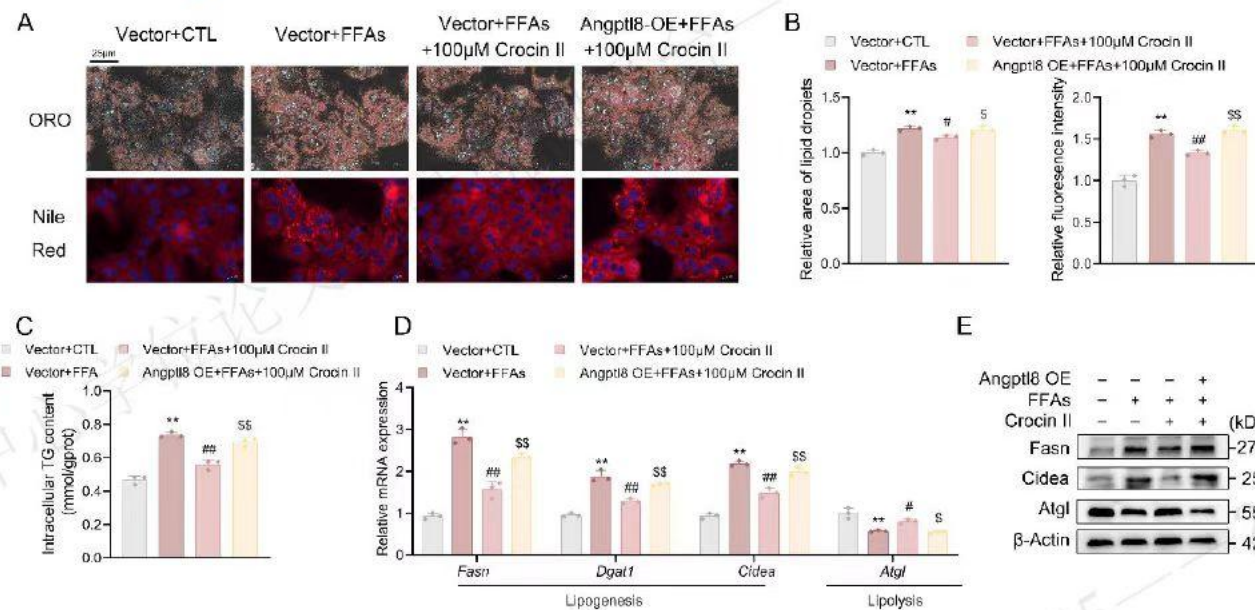
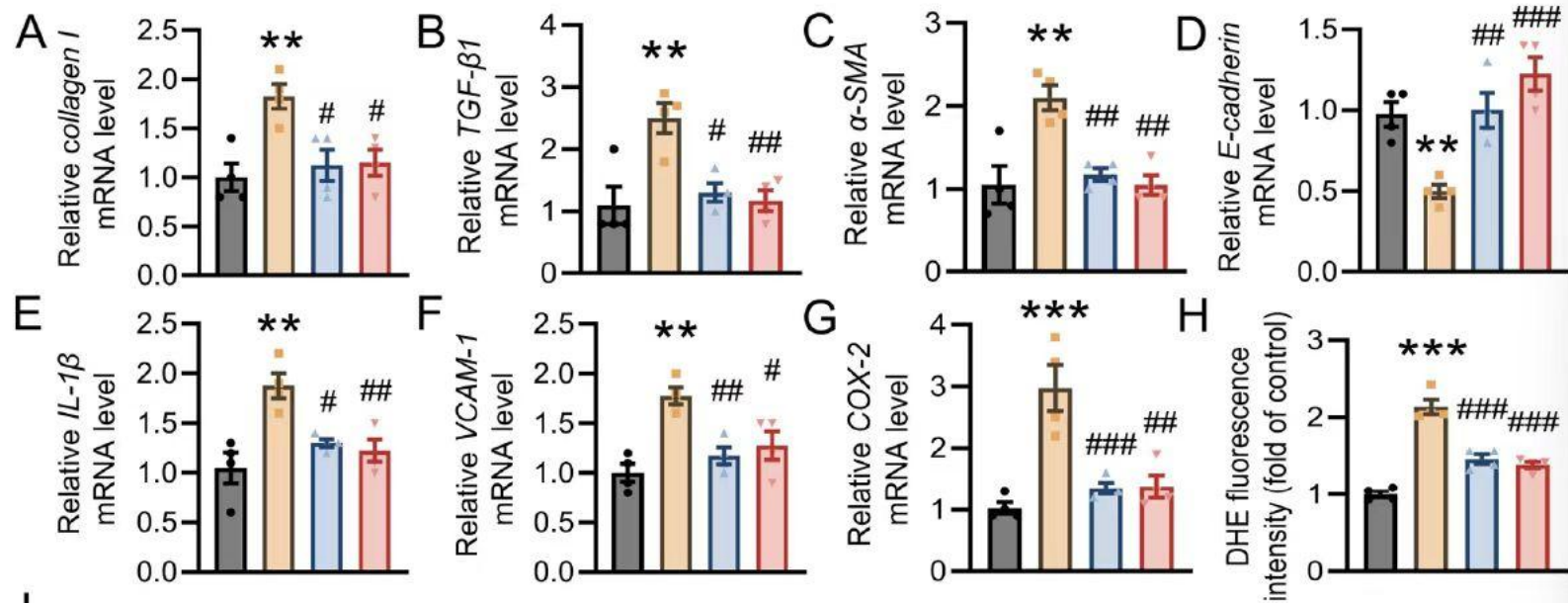


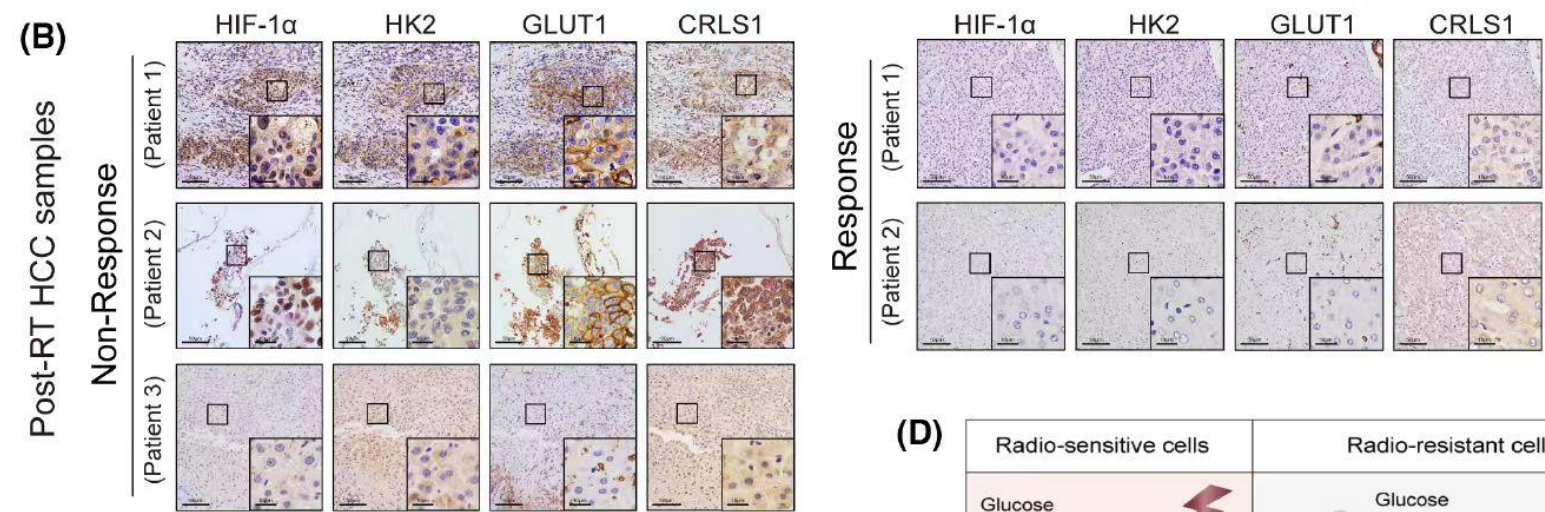
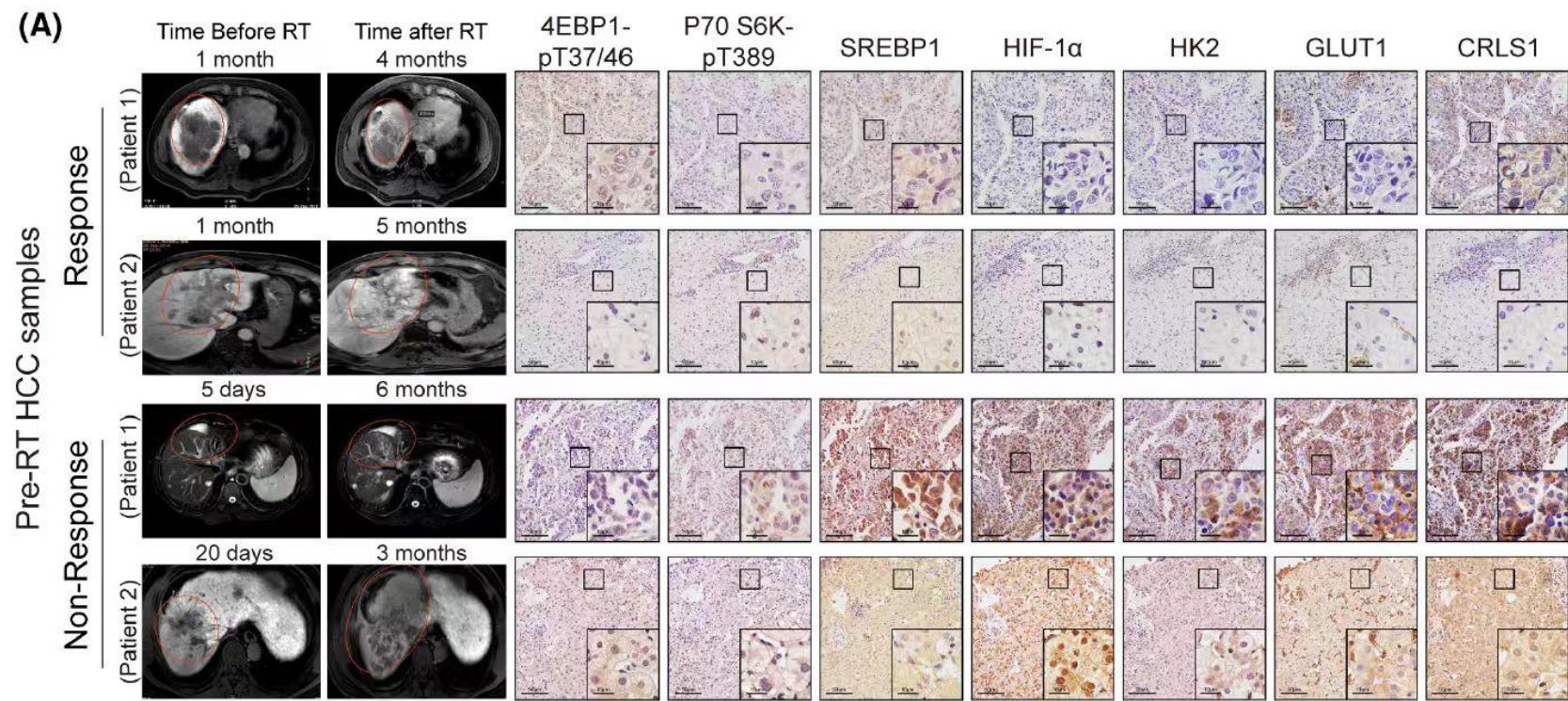






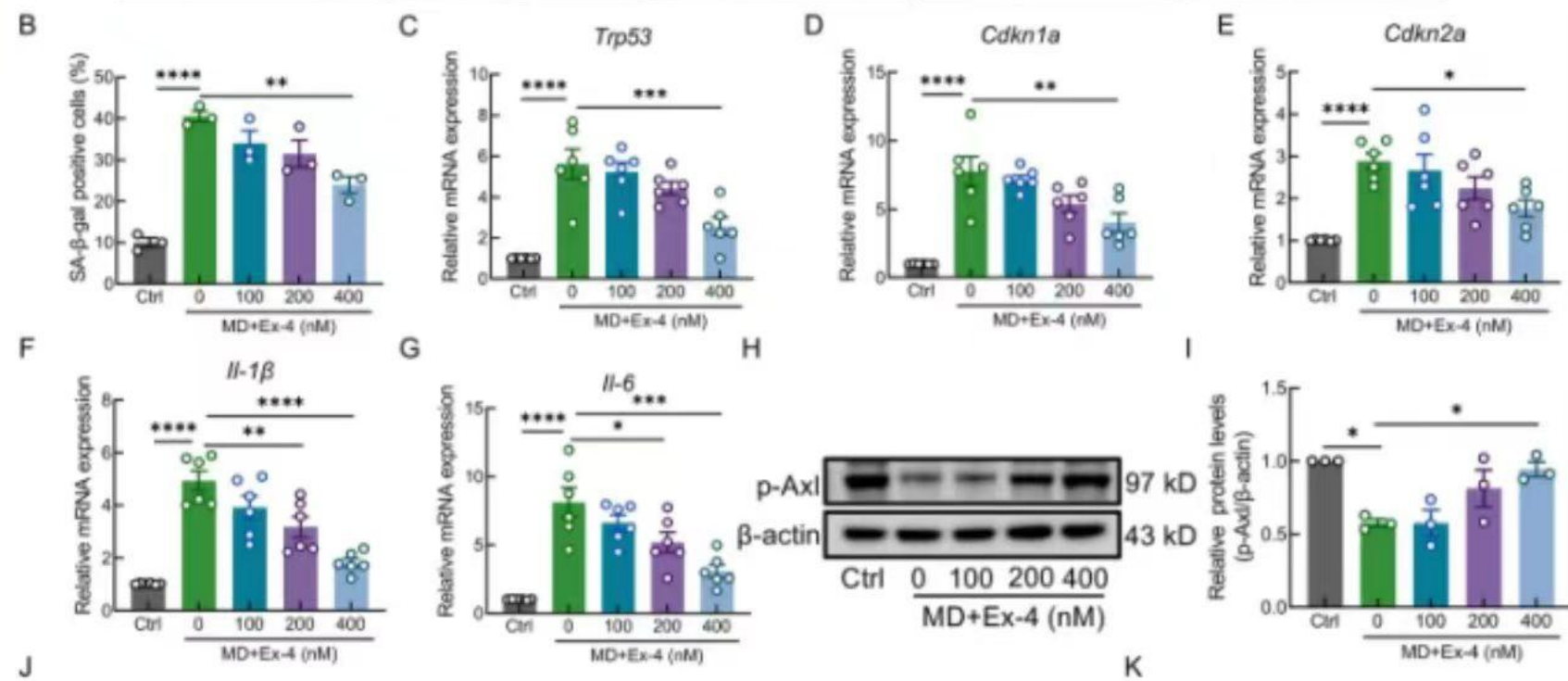


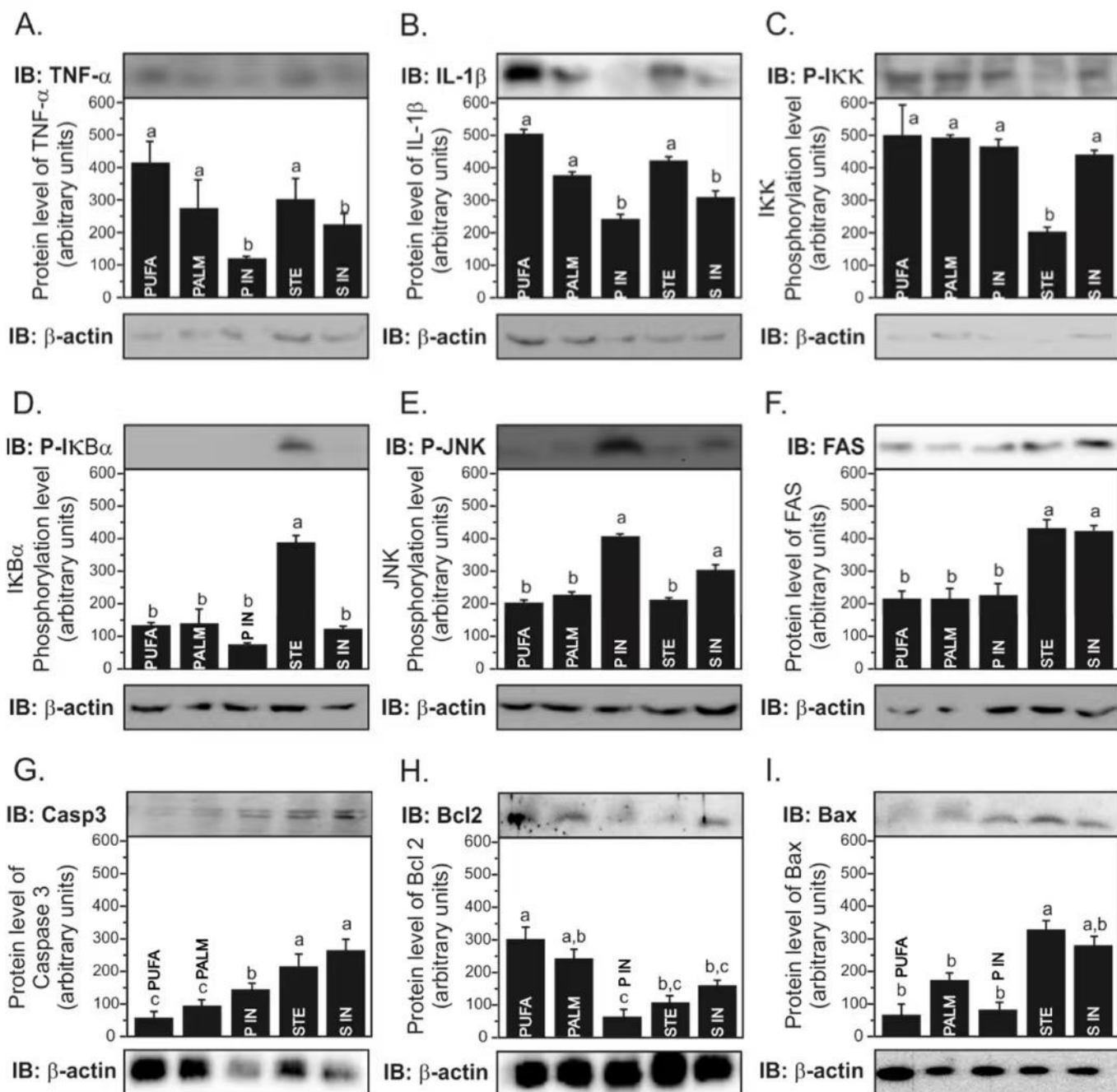
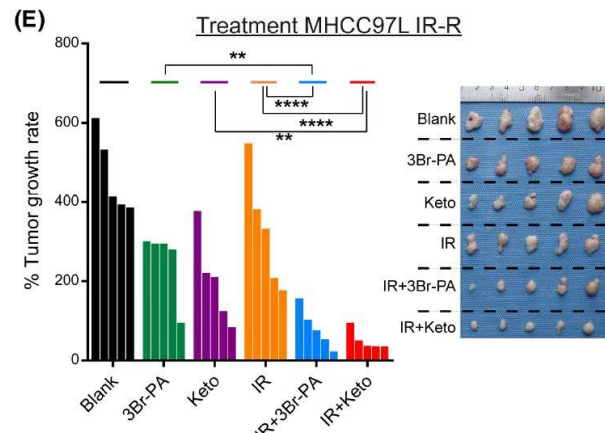
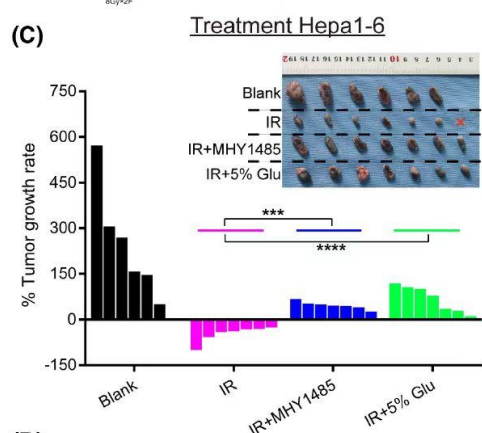
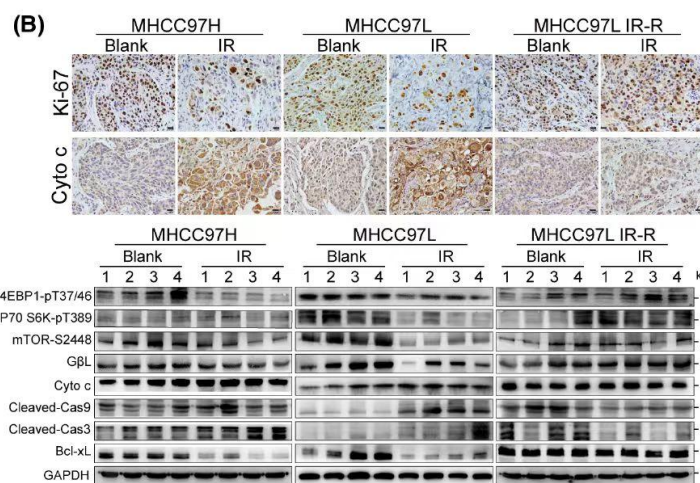
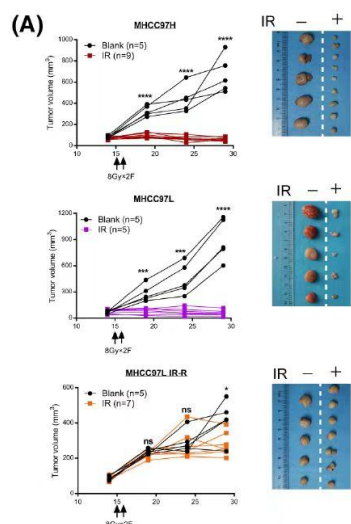


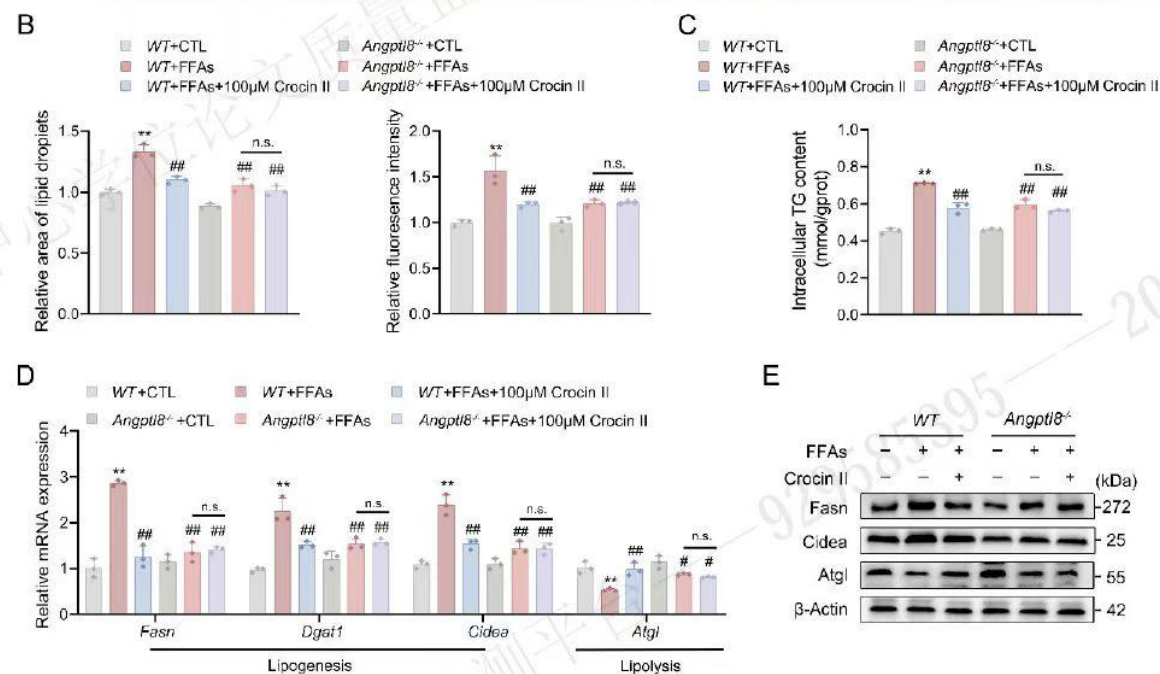
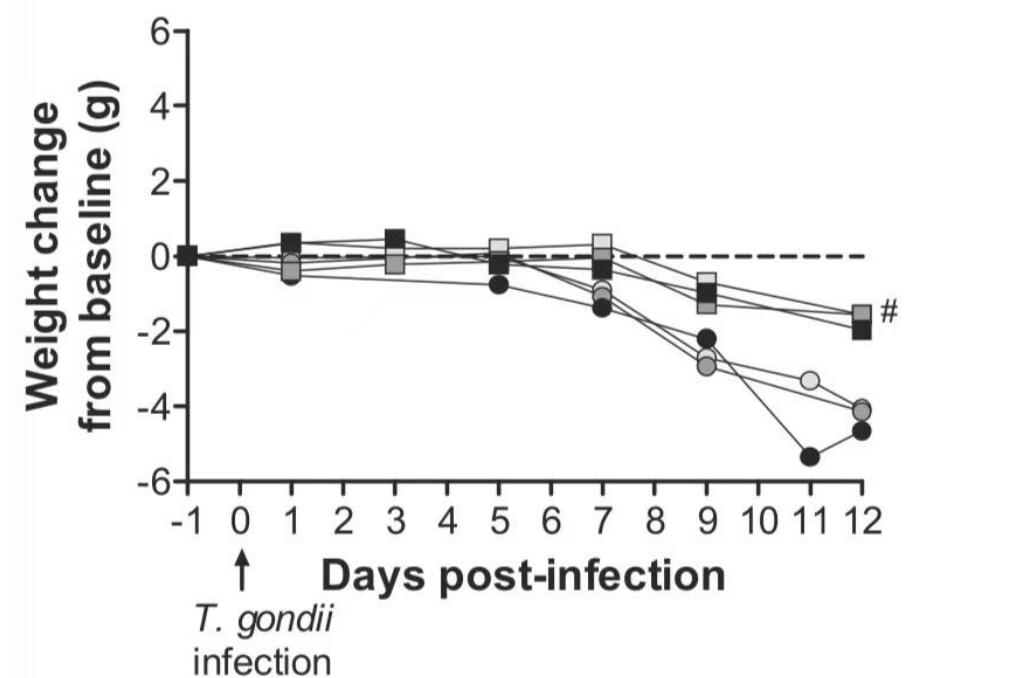
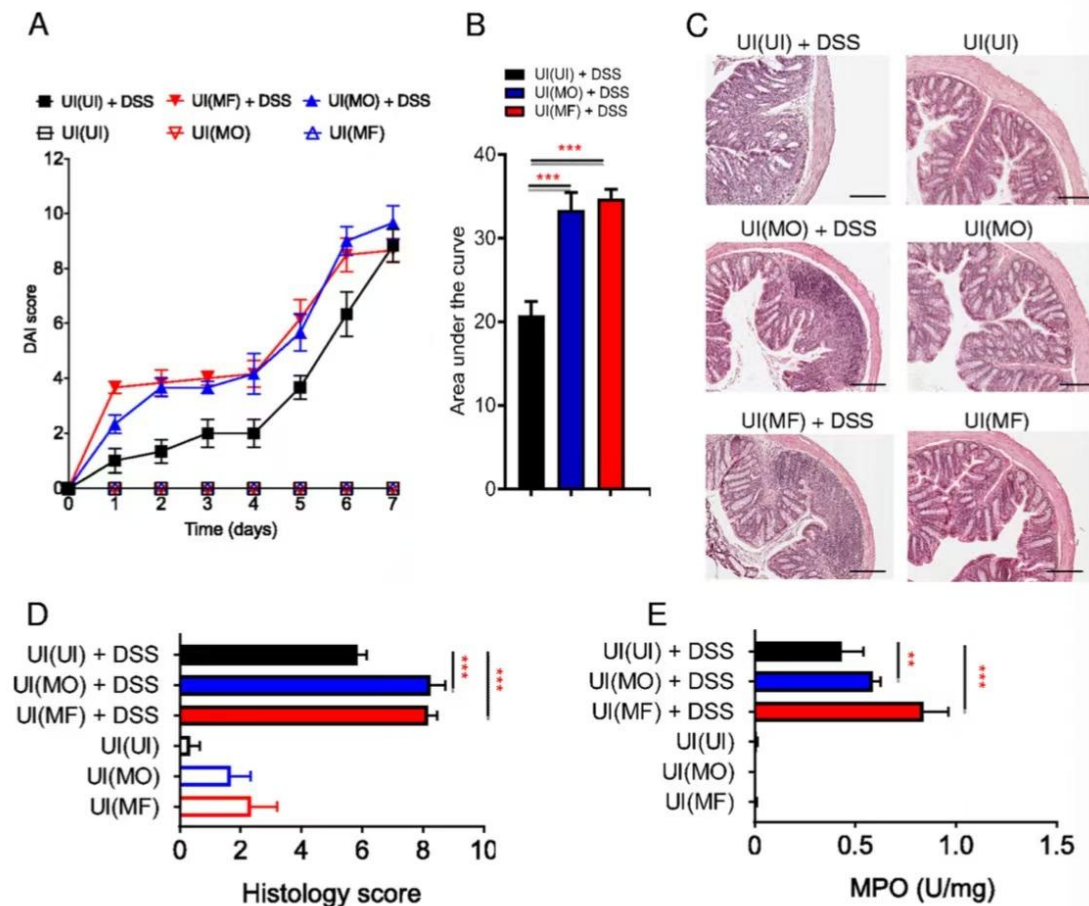


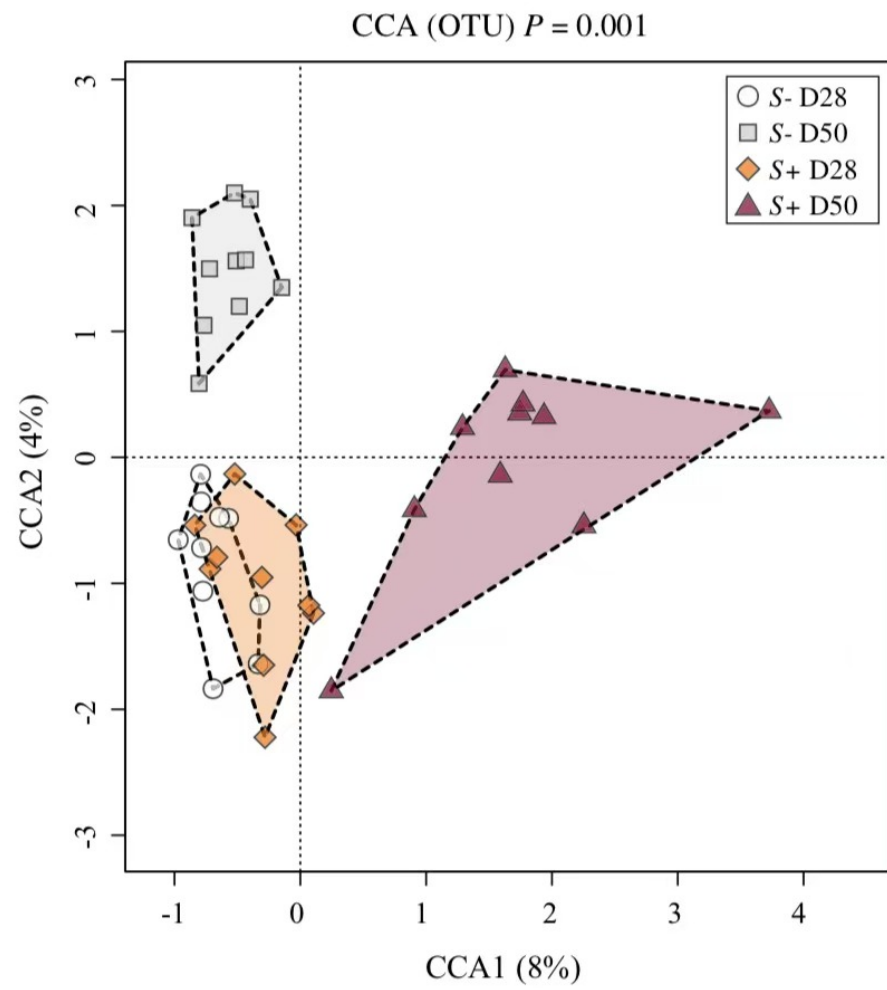
(C) 4EBP1-pT37/46 P70 S6K-pT389 SREBP1 HIF-1 α











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